

MANAGEMENT SERVICES AGREEMENT

DRAFT FOR JONAH BRODSKY / HEALTHCARE COUNSEL REVIEW — NOT LEGAL ADVICE

This is a first prose draft from the NY micro-MSO snowflake spec. It is **not** an attorney work product, **not** a determination of fair market value, and **not** an opinion that any completed form complies with the Education Law, 8 NYCRR Part 29, HIPAA, or any other law. **Jonah Brodsky** reviews and/or revises this template for each customer and bills them directly. Do not treat this file as final until he has worked it.

Do not complete or sign this form if any of the following is true: the fee is or will be a percentage of revenue, collections, visits, referrals, or residual profit; the MSO is a PC or PLLC; clinicians who serve the public will be employed by the MSO; an unlicensed person will be a member, manager, shareholder, director, or officer of the Practice; Path A is selected while the MSO will see patient names, billing, charts, or calendars; or this is being used to paper a practice sale or earnout.

Research used in this draft (counsel confirms): 8 NYCRR 29.1(b)(4); Educ. Law §§ 6509, 6509-a, 6512, 6513; LLCL §§ 411, 1204, 1207; BCL §§ 1507, 1508; NYSED corporate-practice guidance (PC/PLLC may not be a management company); *Advanced Dental of Ardsley, PLLC v. Brown*, 229 AD3d 650 (2d Dept 2024); *Linchitz Practice Mgt., Inc. v Daat Med. Mgt., LLC*, 165 AD3d 908 (2d Dept 2018); 45 CFR §§ 160.103, 164.504(e); [HHS sample BAA provisions](#); State Technology Law § 304 (e-sign) / § 307 (this commercial contract is not an ESRA-excepted will/POA/negotiable instrument); N.Y. GBL Art. 44-A “Freelance Isn’t Free” (if MSO is a one-person organization and compensation is $\geq$ \$800, a written contract must state the pay date and the invoice-submission deadline — this Agreement is intended to be that writing).

Product home: [business-ideas-ny-mso-msa-synthesis.md](#). Spec: [business-ideas-ny-mso-msa-snowflake.md](#)

This Management Services Agreement (this “**Agreement**”) is entered into as of [EFFECTIVE DATE] (the “**Effective Date**”) by and between:

Practice: [PRACTICE LEGAL NAME], a New York [☐ PLLC [☐ PC (check one), with principal office at [PRACTICE ADDRESS — unit exactly as used in formation/CRM], EIN [PRACTICE EIN or “to be inserted”] (the “**Practice**”); and

MSO: [MSO LEGAL NAME], a New York [☐ limited liability company [☐ business corporation (check one; must **not** be a PLLC or PC), with principal office at [MSO ADDRESS], EIN [MSO EIN or “to be inserted”] (the “**MSO**”).

Practice and MSO are each a “**Party**” and together the “**Parties**.”

Recitals

A. Practice is a professional entity authorized to provide licensed professional services in New York. All members and managers of a PLLC, and all shareholders, directors, and officers of a PC (other than entity types this form does not cover), must be persons licensed to practice the profession the Practice is authorized to practice.

B. MSO is a non-professional company. MSO is not a professional corporation or professional limited liability company, does not provide professional services to the public, and is not authorized to practice the profession.

C. Practice wishes to obtain, and MSO wishes to provide, only the non-clinical administrative services checked on **Schedule A**, for the flat fee on **Schedule B**.

D. The Parties intend that clinical control, professional receipts, clinical records, payer contracts, and the hire/fire of licensed professionals remain with Practice, and that compensation to MSO not constitute fee-splitting under the Education Law or 8 NYCRR 29.1(b)(4).

E. Related-party disclosure (check all that apply):

- ☐ One or more owners, members, managers, directors, or officers of Practice have a financial interest in MSO.
- ☐ One or more owners, members, managers, directors, or officers of MSO have a family or household relationship with an owner of Practice.
- ☐ The same natural person will sign this Agreement in two capacities (Practice and MSO).
- ☐ None of the above.

If any box other than “None” is checked, the material facts are: **[NAMES / RELATIONSHIPS / OWNERSHIP %]**. Each Party represents those facts have been disclosed to the persons who must approve related-party contracts for that entity (see LLCL § 411 or BCL analog).

F. These recitals are background only and do not grant MSO any power not granted in the operative sections below.

The Parties agree as follows.

1. Definitions

“**Clinical Staff**” means licensed professionals and other persons who provide or supervise professional services to Practice’s clients or patients.

“**Management Fee**” means the flat monthly dollar amount on Schedule B, and no other formula.

“**Management Services**” means only the items checked on Schedule A, as described there.

“**Non-Clinical Staff**” means persons who do not provide or supervise professional services.

“**PHI**” means protected health information as defined in 45 CFR § 160.103.

2. Engagement; independent contractor; term

2.1 Engagement. Practice engages MSO, and MSO accepts, to perform the Management Services, subject to this Agreement.

2.2 Exclusive or non-exclusive (check one).

- ☐ **Non-exclusive** (default). Practice may obtain similar admin services from others.
- ☐ **Exclusive** as to the **checked** Schedule A lines only. This is not a grant of all administrative control of Practice.

2.3 Independent contractor. MSO is an independent contractor, not a partner, joint venturer, employee, or (except as this Agreement expressly states) agent of Practice. MSO has no authority to bind Practice on clinical, payer, professional-service, or bank-account matters. Classification of any natural person who

performs work is governed by law, not by this label; NY DOL looks at direction and control. If MSO is a one-person organization and the Management Fee is at least \$800, the Parties intend this writing to satisfy GBL Article 44-A: the invoice deadline and payment date are on Schedule B.

2.4 Term. This Agreement begins on the Effective Date and continues for [12] months (the “**Initial Term**”), unless earlier terminated.

2.5 Renewal. The Agreement auto-renews for successive 12-month periods unless a Party gives written notice of non-renewal at least [30] days before the then-current term ends. Renewal does not change the Management Fee unless Schedule B is amended in a signed writing.

2.6 Termination for convenience. Either Party may terminate for any reason by written notice at least [30] days in advance.

2.7 Termination for cause. Either Party may terminate if the other materially breaches and does not cure within [15] days after written notice describing the breach. Breach includes (without limitation) MSO exercising reserved Practice authority under Section 4, a prohibited fee formula, or MSO unilateral control of Practice’s operating account.

2.8 Immediate termination. Either Party may terminate immediately by written notice if: (a) Practice’s (or its sole licensee’s) license to practice is suspended, revoked, surrendered, or not renewed; (b) a Party becomes insolvent, makes an assignment for creditors, or has a bankruptcy or receivership proceeding filed by or against it that is not dismissed within 30 days; or (c) a Party is enjoined from performing this Agreement.

2.9 Effect of termination. MSO stops Management Services on the termination date. Practice pays any **earned unpaid Management Fee**, including a **pro-rata** amount for a partial month equal to:

$(\text{Management Fee}) \times (\text{number of calendar days from the later of the start of that month or the Effective Date through the termination date, inclusive}) \div (\text{number of calendar days in that month}).$

MSO has no right to any percentage of Practice receipts, including after termination. Within 5 business days, MSO shall return or irretrievably delete Practice confidential information and (if Path B) PHI as the BAA requires, and shall cut off its access to Practice systems, calendars, and accounts. Practice shall not condition release of clinical records on payment; records remain Practice property.

3. Services (Schedule A)

3.1 Scope. MSO shall perform only checked Schedule A items. Unchecked items are out of scope even if discussed orally or in marketing. There is no catch-all for “other services reasonably required.” Custom work is in scope only if written on Schedule A.

3.2 Changes. The Parties may add or remove services only by signed amendment to Schedule A. The Management Fee does not change unless Schedule B is also amended in a signed writing, and any new fee must remain a flat dollar amount (or a flat step-up table), not a function of receipts.

3.3 Standard of admin work. MSO shall perform Management Services in a commercially reasonable manner. MSO does not practice a profession and does not give legal, tax, or clinical advice.

4. Exclusions and clinical authority

4.1 MSO shall not: diagnose, treat, or provide psychotherapy or other professional services; write or impose clinical protocols; supervise Clinical Staff in the practice of the profession; set or change fees charged to clients or patients for professional services; direct referrals or payer mix as a clinical or volume matter; hire or fire Clinical Staff or decide clinical competency; own or hold hostage clinical records; or sign payer contracts or professional-service agreements on Practice's behalf.

4.2 Reserved to Practice. Practice retains exclusive authority over professional judgment and standard of care; hire, fire, and clinical-competency decisions for Clinical Staff; ownership and control of clinical records; professional fee schedules; and execution of payer and professional-service contracts.

4.3 Narrow edge. MSO may recruit **candidates** for clinical roles (Practice decides whether to hire); recommend vendors, software, or non-clinical workflows (Practice decides clinical use); and prepare drafts for Practice's signature (only Practice signs).

4.4 Conflict rule. If any other clause of this Agreement conflicts with this Section 4, this Section 4 controls. Nothing here authorizes MSO to practice a profession or to act as a PC or PLLC.

4.5 Intent (not a legal opinion). The Parties intend to comply with restrictions on unauthorized practice, unauthorized use of professional titles, corporate practice of the professions, and fee-splitting, including 8 NYCRR 29.1(b)(4), as they apply to PLLCs and PCs. This sentence is an expression of intent, not an opinion of counsel or of any formation company that supplied a form.

5. Compensation (Schedule B)

5.1 Flat fee only. Practice shall pay MSO the Management Fee on Schedule B. The Management Fee is a stated dollar amount for Management Services. It is **not** a percentage of gross or net revenue, collections, or profits; **not** per-visit, per-patient, or per-referral; and **not** a residual claim on Practice cash after expenses.

5.2 Invoices and payment. MSO shall submit each invoice by the invoice deadline on Schedule B. Practice shall pay by the payment date on Schedule B, by transfer from a Practice-named account to an MSO-named account.

5.3 Late charges. Late charges, if any, are only as stated on Schedule B and may be calculated **only** on unpaid Management Fee dollars, never on Practice collections or receipts.

5.4 Pass-throughs. None, unless listed at cost on Schedule B. A pass-through may not be a percentage of receipts.

5.5 No FMV opinion. The Parties believe the Management Fee is payment for the listed admin work. Neither this form nor any non-attorney who prepared it opines that the amount is fair market value. The Parties may obtain independent valuation if they wish.

6. Financial separation

6.1 Practice account. Practice shall maintain at least one deposit account in Practice's legal name. Client payments, insurance, and professional fees shall be deposited to Practice accounts. Authorized signers on Practice's operating account shall be licensed owners (members/shareholders) or licensed officers of Practice only. MSO shall not be a signer with unilateral control of that account.

6.2 MSO account. MSO shall maintain its own account. The Management Fee is paid from Practice to MSO as a vendor payment. MSO shall not receive professional receipts as MSO revenue.

6.3 Prohibited. Daily or standing sweep of Practice funds into an MSO-only account; MSO unilateral control of Practice's operating account; commingling of professional receipts with MSO operating funds.

6.4 Books. Each Party keeps its own books. Practice may inspect MSO records reasonably needed to confirm the Management Fee (and any listed pass-throughs) was billed and paid as agreed. Tax characterization is for the Parties' accountants.

7. Personnel and operations

7.1 Clinical Staff. Clinical Staff who serve the public are employed or contracted by **Practice**, not MSO. MSO shall not supervise professional services or set Clinical Staff compensation.

7.2 Non-Clinical Staff (check one).

- ☐ Practice employs (or will employ) Non-Clinical Staff; MSO does not employ them.
- ☐ MSO employs specified Non-Clinical Staff: **[ROLES]**. MSO may direct their **non-clinical** work. Practice still owns client relationships and clinical calendar priorities.
- ☐ No Non-Clinical Staff at signing.

7.3 Operations match paper. The Parties shall operate consistently with this Agreement. If operations drift (for example MSO controlling clinical hiring or the Practice operating account), the Parties shall amend this Agreement or unwind; they shall not "fix it in practice" while leaving this paper unchanged.

7.4 Policies. MSO may propose non-clinical HR, IT, and vendor policies. Clinical policies, informed consent, and standard of care remain Practice's.

7.5 Public statements. Public statements about professional services shall attribute those services to Practice and named licensees. MSO shall not hold itself out as able to practice the profession or use a restricted professional title. Directory and Google Business listings for the professional practice shall use Practice's legal name and licensed professionals, not an unlicensed person as "owner" or "therapist" of the practice.

8. Records; PHI; confidentiality

8.1 Clinical records. Practice owns all clinical and treatment records. MSO may access them only as needed to perform checked Management Services, if at all, and only under Section 8.2.

8.2 PHI fork (check exactly one).

- ☐ **Path A.** MSO will **not** create, receive, maintain, or transmit PHI for Practice. Schedule A does not include bookkeeping or billing that identifies patients, EHR or IT admin with chart access, or scheduling that uses patient names. No BAA is attached. If this box is inconsistent with Schedule A, Path A is void and the Parties must execute a BAA before MSO continues those services.
- ☐ **Path B.** MSO will create, receive, maintain, or transmit PHI in performing Management Services. A business associate agreement is attached as **Exhibit C** (use counsel's BAA or provisions meeting 45 CFR § 164.504(e); HHS publishes sample provisions). The BAA controls as to PHI. MSO shall use minimum necessary PHI.

8.3 Confidentiality. Each Party shall keep the other's non-public business information confidential, except for disclosures to professional advisors, as required by law, or that is or becomes public other than by breach. This Section 8.3 survives [3] years after termination. PHI duties survive as the BAA and law require.

9. Indemnity; insurance; limitation

9.1 MSO indemnity. MSO shall indemnify Practice against third-party claims and reasonable defense costs to the extent caused by MSO's negligent performance of Management Services or MSO's breach of this Agreement.

9.2 Practice indemnity. Practice shall indemnify MSO against third-party claims and reasonable defense costs to the extent caused by Practice's professional services or clinical care.

9.3 Carve-out. Section 9.1 and 9.2 do not require a Party to indemnify the other for that other Party's willful misconduct or knowing violation of law (including knowing fee-splitting or unauthorized practice).

9.4 Insurance. Practice shall maintain professional liability insurance covering clinical services. MSO shall maintain commercial general liability insurance. If Path B, MSO shall also maintain cyber/privacy liability insurance in commercially reasonable amounts. Certificates on request.

9.5 Limitation of liability (optional — check with counsel).

- ☐ **Omitted** (default for this draft).
 - ☐ Neither Party is liable to the other for consequential or lost-profit damages, except for (i) indemnity obligations in this Section 9, (ii) breach of confidentiality or the BAA, and (iii) willful misconduct. This cap shall not shift malpractice to MSO or an MSO HIPAA breach to Practice.
-

10. General

10.1 Governing law; venue. New York law governs, without regard to conflict-of-law rules. The exclusive venue (place of trial) is the Supreme Court of the State of New York, County of [VENUE COUNTY — county of Practice's principal office]. "Courts of the State of New York" without a county is not sufficient.

10.2 Entire agreement; no side letters. This Agreement, Schedule A, Schedule B, the related-party disclosure in the recitals, and Exhibit C (if Path B) are the entire agreement. There are no oral or side arrangements for other compensation (including any percentage of receipts). Amendments must be in a writing signed by both Parties in the capacities in Section 10.7.

10.3 Notices. Notices under this Agreement are effective when sent by email with a copy by overnight courier, or three business days after mailing by certified mail, to:

Practice: [EMAIL] / [PHYSICAL]

MSO: [EMAIL] / [PHYSICAL]

10.4 Assignment. Neither Party may assign this Agreement without the other's prior written consent, except that a Party may assign to a successor entity of the **same type** (Practice: successor PLLC or PC owned only by licensed professionals; MSO: successor ordinary LLC or corporation) under the same beneficial ownership, on written notice. Practice's duties may not be assigned to a non-professional company.

10.5 Severability; waiver. If a provision is held invalid, the rest remains in effect. The Parties do not intend severability to save a percentage-of-receipts fee; that formula is not used. A failure to enforce a right once is not a waiver.

10.6 Survival. Sections 4, 5.1, 6, 8, 9, 10.1, and earned unpaid Management Fee obligations survive termination.

10.7 Counterparts; e-sign; capacity. This Agreement may be signed in counterparts, including electronic signatures, which have the effect of wet-ink signatures under the Electronic Signatures and Records Act (this Agreement is not an instrument excepted by State Technology Law § 307). Each signer represents they are authorized to bind the entity for which they sign. **The same natural person, if signing for both Parties, must sign twice**, once in each capacity below.

10.8 Construction. “Including” means “including without limitation.” Headings are for convenience.

Signatures

PRACTICE
[PRACTICE LEGAL NAME], a New York [PLLC / PC]

By: _____
Name: [LICENSED OWNER NAME]
Title: [Member / President / other licensed officer]
Date: _____
(Sign only for Practice.)

MSO
[MSO LEGAL NAME], a New York [LLC / corporation]

By: _____
Name: [MSO SIGNER NAME]
Title: [Member / Manager / President]
Date: _____
(Sign only for MSO.)

Schedule A — Management Services

Check **only** services MSO will actually perform. For each checked line, complete deliverable, cadence, and MSO contact.

#	Service	Y/N	Deliverable (one line)	Cadence	MSO contact
1	Bookkeeping / accounting (non-clinical)	[]		monthly / other: _____	
2	Payroll/benefits admin for Non-Clinical Staff only	[]			
3	HR admin (onboarding/PTO paperwork; not clinical competency)	[]			

#	Service	Y/N	Deliverable (one line)	Cadence	MSO contact
4	Facilities / lease administration (MSO is not landlord under this form)	☐			
5	IT / EHR administration (not clinical documentation decisions)	☐			
6	Marketing / website / social (clinical claims attributed to Practice)	☐			
7	Scheduling systems / front-desk ops (Practice sets clinical priorities)	☐			
8	Credentialing / payer-enrollment support (Practice signs)	☐			
9	Compliance support (calendars/checklists; not legal advice)	☐			
10	Custom (no catch-all): ____	☐			

If any of rows 1, 5, 7 (with patient names), or billing-like bookkeeping is Yes, **Path B + Exhibit C** is required.

Schedule B — Management Fee

Item	Value
Monthly Management Fee (USD, no percentage)	\$
Invoice submission deadline (FIFA: date by which MSO must invoice)	Day of each month (or “end of month served”)
Payment date	Day of each month / within [15] days after invoice
Method	☐ Invoice then pay ☐ Auto-pay from Practice account Practice controls
Late charge (optional)	☐ None ☐ % per month on unpaid Management Fee only , not on collections
Pass-throughs at cost	☐ None ☐ Listed: [item / cap]
Practice bank (name / last4) — professional receipts	
Practice operating-account signers (licensed owners/officers only)	
MSO bank (name / last4)	

Exhibit C — Business Associate Agreement (Path B only)

Attach a BAA that contains the elements of 45 CFR § 164.504(e). Counsel may start from [HHS sample BAA provisions](#). This draft does **not** include a homemade BAA.

If Path A, write “**Not attached — Path A**” and do not send PHI to MSO.